HEALTHCARE ANALYTICS / EXECUTIVE BRIEF

Medicaid prescription spending
From dollars to review priorities

Mohammed Mia | Python + SQL | 2024

$27.54B
Reported reimbursement
Before manufacturer rebates

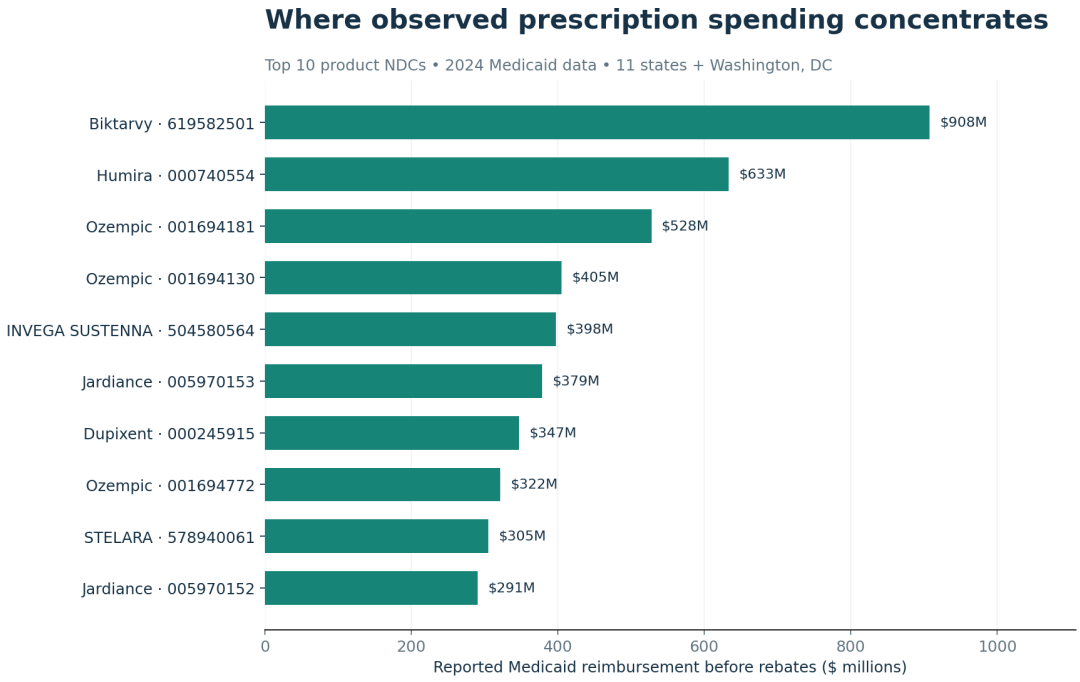
12
Jurisdictions analyzed
11 states + Washington, DC

37.1%
Top 50 product share
Of reported reimbursement

The question I wanted to answer

Where does Medicaid prescription spending concentrate, and which products deserve closer price review? I combined utilization records with FDA product information and followed five questions: product spending, pharmacologic categories, price differences, recorded alternatives, and geographic/reporting context.

Q1 / I started with the products behind the dollars



Product-level NDCs combine package sizes, not all strengths of a brand. Suppressed reimbursement is unavailable.

Biktarvy product NDC 619582501 contributes $907.89M. The top 10 product NDCs account for 16.4% of reported reimbursement, and the top 50 account for 37.1%. I would begin detailed review with this concentrated product set.

A product NDC combines package sizes, not every strength of a brand. This is a product-level ranking within the included extract.

Q2 / PHARMACOLOGIC CONTEXT

Understanding the categories behind individual products

Product rankings identify dollar exposure. I then use the directory's recorded established pharmacologic class (EPC) tags to understand the categories represented among those dollars.

The pharmacologic categories behind the dollars



Tags overlap: one product can contribute to multiple bars. Do not add these amounts together.

Products with the GLP-1 Receptor Agonist tag account for $2.94B. HIV-related reverse transcriptase inhibitors, atypical antipsychotics, and SGLT2 inhibitors also appear prominently. Another $5.19B has no recorded EPC tag or unmatched metadata; those dollars remain in overall spending.

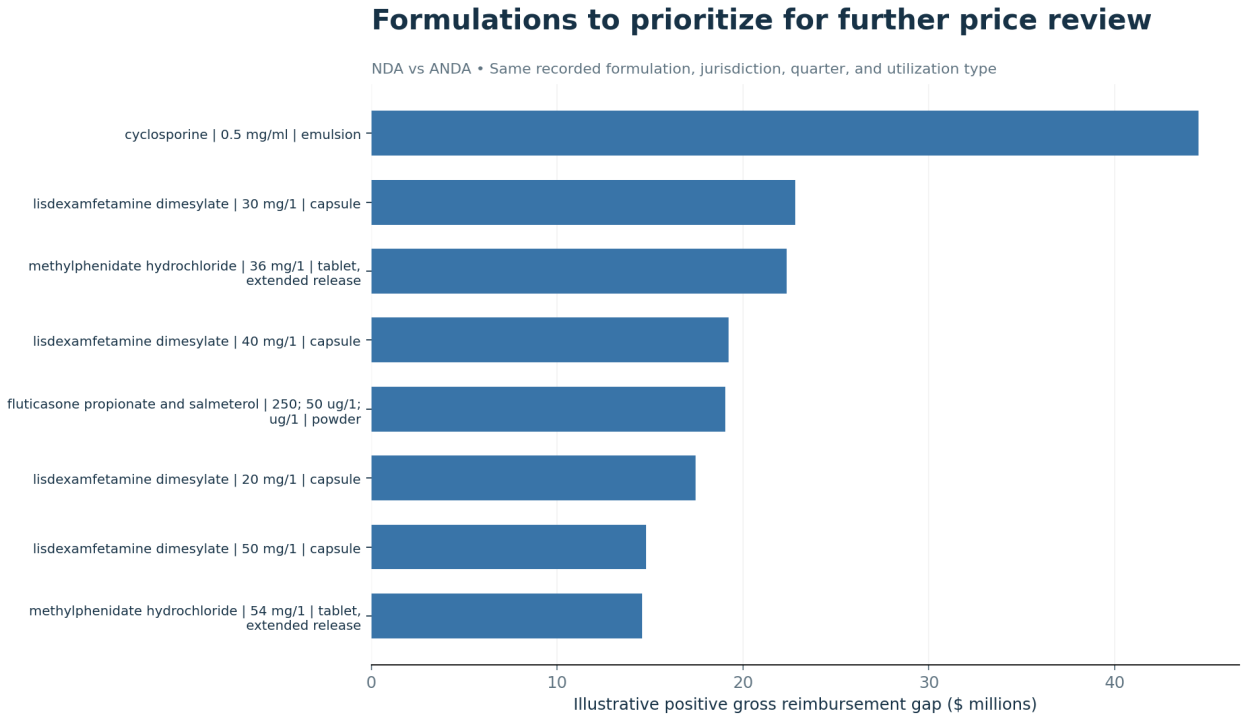
A product can carry multiple tags. I do not add class totals together or interpret them as mutually exclusive budget shares.

Q3 / How I move from spending to price review

I compare NDA- and ANDA-listed prescription products within the same recorded formulation, jurisdiction, quarter, and utilization type. For each side I calculate total reimbursement divided by total units. This gives utilization its appropriate weight when the question concerns observed dollars.

The matched comparison covers 562 formulations and 6,675 reporting groups. I use the observed price difference at NDA utilization to identify where a closer review could be worthwhile.

Turning price differences into a focused review list



Screening only: therapeutic equivalence, unit type, rebate differences and feasibility are not verified. Not net savings.

Positive differences sum to a $516.16M illustrative gross reimbursement gap. Including negative differences gives a $506.73M signed aggregate. I calculate the positive amount within each reporting group before aggregating, and retain both measures.

The leading recorded formulation is cyclosporine ophthalmic emulsion at 0.5 mg/mL, with a $44.50M positive gap. Lisdexamfetamine dimesylate oral capsules at 30 mg and methylphenidate hydrochloride oral extended-release tablets at 36 mg have gaps of $22.83M and $22.34M.

Q4 / What a recorded match does and does not establish

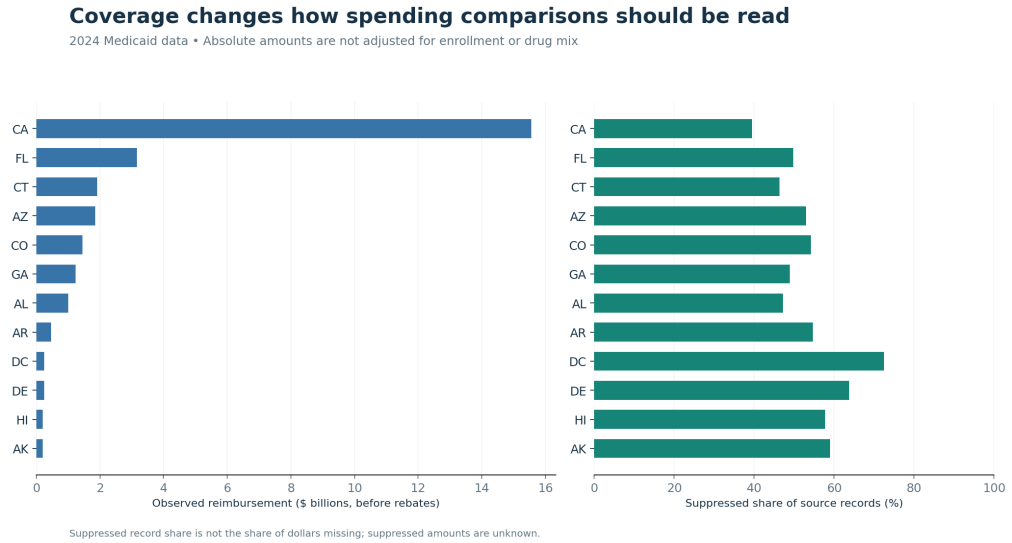
I retain ingredient, strength, strength unit, dosage form, and route in the matching rules. The cyclosporine result applies to that specific recorded formulation, not every product containing cyclosporine.

An exact descriptor match still needs verification of reimbursement units, applicable therapeutic-equivalence evidence, product availability, patient suitability, and net rebate economics. NDA and ANDA categories do not fully define brand/generic status.

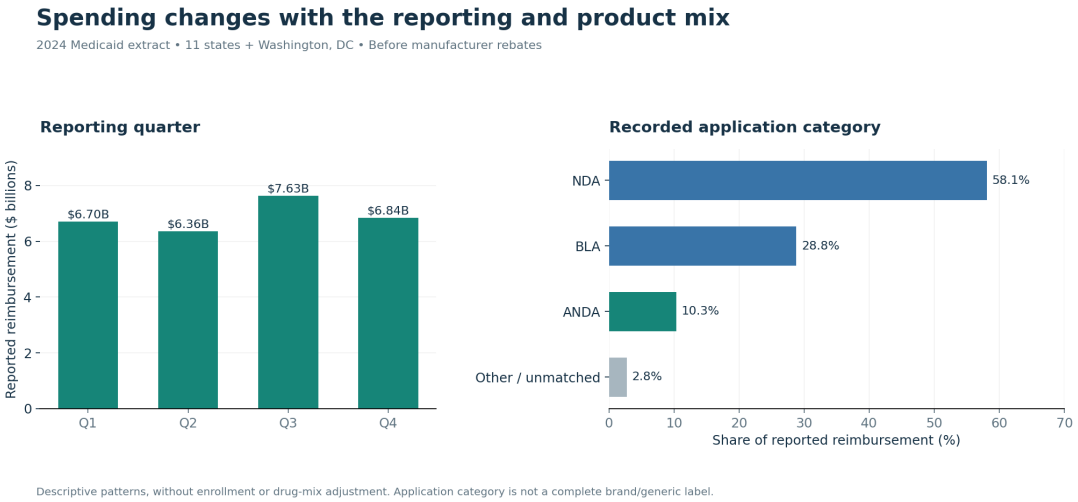
My conclusion: these are investigation candidates. The gross gap is not a forecast of achievable or net savings.

Q5 / GEOGRAPHIC AND REPORTING CONTEXT

Reading the findings in their reporting context



California contributes 56.5% of this dataset's reimbursement. I interpret that as absolute spending concentration. Enrollment, patient and drug mix, utilization, and reporting completeness are not controlled. Suppressed record share is not missing dollar share.



Q3 has the highest reported reimbursement at $7.63B. NDA, BLA, and ANDA listings account for 58.1%, 28.8%, and 10.3% of spending. These distributions describe the extract; they do not establish causal drivers.

The decision I would take forward

I would prioritize high-dollar products, organize the review with pharmacologic context, and investigate exact recorded formulation matches. I would validate comparability and net economics before evaluating an intervention.

Scope: 1,048,575 utilization records, including 543,945 with suppressed amounts. The extract reaches the worksheet row limit; complete national or within-jurisdiction coverage is not established. The directory is not a point-in-time 2024 snapshot.

Sources: CMS 2024 utilization data, FDA NDC Directory. Repository, notebook, and methodology.

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